INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cisplatin + etoposide with radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00279
Version 8

Reference intravenous (IV) plan: four 21-day cycles. Day 1: cisplatin + etoposide. Days 2–3: etoposide. Radiation follows its own appointments. Capsules, split cisplatin, carboplatin substitution or 28-day cycles require a different personalised prescription.

Treatment stages

01 **Four chemotherapy cycles**
Days 1–3 each cycle

02 **Radiotherapy alongside chemotherapy**
Separate radiotherapy (RT) appointments

03 **Recovery and response review**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Cisplatin + etoposide with radiotherapy · National Cancer Control Programme (NCCP) 00279

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

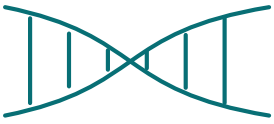
My chemotherapy medicines

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

PLATINUM CHEMOTHERAPY

Cisplatin

Platinum chemotherapy



Damages deoxyribonucleic acid (the cell's genetic material) (DNA)

Cisplatin forms links in DNA that interfere with cell copying. Fluids and kidney checks help reduce harm to healthy tissue.

Treatment reaches cells → Damages DNA → Cancer growth may slow

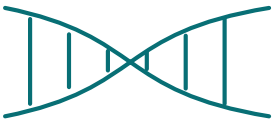
What to watch for

Report poor fluid intake, reduced urine, ringing in the ears or numbness.

CHEMOTHERAPY

Etoposide

Chemotherapy



Disrupts DNA repair during cell division

Etoposide blocks an enzyme cells need while copying DNA. The route and prescribed amount differ between infusion and capsules; they are not interchangeable milligram for milligram.

Treatment reaches cells → Disrupts DNA repair during cell division → Cancer growth may slow

What to watch for

Report fever, bleeding, severe mouth soreness or vomiting.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why chemotherapy and radiotherapy are combined

Studies support combining systemic treatment with radiation in the relevant lung-cancer setting.

The evidence comes from specific treatment schedules and patient groups. Your team explains how it applies to your situation; study averages cannot predict an individual outcome.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Cisplatin + etoposide with radiotherapy · NCCP 00279

My treatment overall

Treatment stages

01 **Four chemotherapy cycles**
Days 1–3 each cycle

02 **Radiotherapy alongside chemotherapy**
Separate radiotherapy (RT) appointments

03 **Recovery and response review**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAYS 1–3 EACH CYCLE

Four chemotherapy cycles

1 2 3 4

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

SEPARATE RT APPOINTMENTS

Radiotherapy alongside chemotherapy

Team-agreed

Usually begins with cycle 1; later start may be appropriate. The radiation team confirms dose, number of visits and timing. A chemotherapy delay does not automatically mean radiation stops.

NEXT PLAN AGREED

Recovery and response review

Review

After treatment, your team checks recovery, persistent symptoms and response. Scan type/timing and any useful tumour-marker testing are individual decisions; this regimen does not specify a universal scan or marker timetable.

The chemotherapy course is finite; radiotherapy timing and next treatment are confirmed separately.

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Required safety tests are distinct from team-agreed imaging and follow-up; no universal tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00279 v8 · official regimen

THE WHOLE TREATMENT JOURNEY

Cisplatin + etoposide with radiotherapy: treatment and checks

Limited-stage small-cell lung cancer · combined chemotherapy and radiation

Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy NCCP 00279	Confirm aim and prescription	Days1–3 each cycle	No additional routine doses	Repeat up to4cycles	Next treatment only if agreed
Radiotherapy NCCP 00279	Separate planning appointment	Usually starts cycle1	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests NCCP 00279	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods before cycle	Follow abnormal results
Symptoms / response NCCP 00279	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

What each check is for

1 **Prepare**

Before first dose

Chemotherapy and radiotherapy work together to treat limited-stage small-cell lung cancer. Your team explains the aim and why this combined treatment is suitable for you.

Ask / act: Confirm prescription, radiation plan and contacts.

2

Monitor

Each cycle

Check fitness and tolerability

Ask / **act:** Report symptoms and confirm test results before dosing.

3



Review

After course / when clinically indicated

Assess response and recovery

Ask / **act:** Confirm the next plan and persistent symptoms.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Confirm your plan

Reference intravenous (IV) plan: etoposide first on Days 1–3, cisplatin after etoposide on Day 1, each infusion about one hour plus checks and fluids. If prescribed, Days 2–3 etoposide may instead be capsules twice daily. Confirm route, platinum, interval and hydration before treatment; never combine alternatives yourself.



STEP 2

Keep the contact card

The team supplies the daytime and out-of-hours numbers. Keep the card with you and show it when seeking medical care.



STEP 3

Two linked treatment teams

Confirm which team to contact for radiation or chemotherapy changes. Report swallowing problems early and ask about transport, nutrition and fatigue support.

Symptoms and supportive care

Cisplatin + etoposide with radiotherapy · NCCP 00279

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cisplatin + etoposide	 **Day 1** Reference Day 1 intravenous (IV) treatment plus hydration.
Etoposide Days 2–3	 **Days 2–3** IV each day, or prescribed oral alternative. Split cisplatin only if explicitly prescribed.
Symptoms and separate RT plan	 **Days 1–21** Symptoms may occur on any day. This is not a radiation-fraction schedule or a precise low-blood-count curve.

Relative schedule, not personal dates: Reference plan: four 21-day cycles. Day 1: cisplatin and etoposide. Days 2–3: etoposide. Radiotherapy usually starts with cycle 1; its appointments are planned separately. Capsules, split cisplatin, a 28-day interval or a different platinum are used only if explicitly prescribed.

No fixed symptom day: Sickness, fatigue, low counts, swallowing problems or breathing symptoms can occur between visits and after treatment. Do not wait for a calendar window to seek advice.

Radiation is a separate schedule: Chemotherapy Day 1/Day 2/Day 3 or weekly counts are not radiation fractions. The radiation oncology team supplies the exact RT timetable.

Why these bands stay broad: Reference cycle without patient dates. RT visits and dose are separately prescribed; the symptom band is not a nadir prediction. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00279 v8 · official regimen ↗

There are no guaranteed "safe days". Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Between-treatment notes

Day 1: Day 1: reference combination

Blood results, symptoms, anti-sickness medicines and hydration are checked before both drugs. Tell the nurse about any hearing, nerve or fluid-intake problems.

Day 2–3: Days 2–3: etoposide

IV treatment each day or prescribed capsules twice daily. The split-cisplatin alternative changes these visits. Use your written prescription; do not add IV and oral etoposide together.

Day 4–7: Support and recovery

No routine chemotherapy in the reference plan. Follow ongoing anti-sickness medicines and separate RT appointments. Call early for poor intake, vomiting, swallowing pain or fever.

Day 8–21: Between chemotherapy cycles

Fatigue, mouth symptoms and infection risk can persist. RT may continue according to its own plan. Report symptoms when they occur; do not wait for the next blood test.

Support around the medicines

1 Fluids and sickness prevention

Cisplatin needs a planned fluid and anti-sickness course. Follow the labelled medicines for each treatment day and the days afterwards. Call if vomiting prevents drinking or taking medicines.

2 If capsules are prescribed

Etoposide capsules on Days 2–3 replace the IV doses on those days. Take the prescribed amount twice daily on an empty stomach. Follow pharmacy handling instructions and contact the team if a dose is missed or vomited; do not take an extra dose without advice.

3 Radiotherapy support

Ask about swallowing, eating, skin care and fatigue during radiation. Painful swallowing or poor intake needs early help; inability to drink needs urgent assessment.

Questions worth asking

Which drugs and routes are prescribed for me?

Confirm the exact drug names, routes and interval against your written prescription. The team must explain any change from this reference plan.

How do my radiation appointments fit around chemotherapy?

Radiation has its own calendar. Ask for the combined plan and whom to contact if a chemotherapy or radiation visit changes.

Which blood tests are required before the next dose?

Use the checks table as a prompt. Your team confirms which tests are needed, where they are done and who reviews the results before treatment.

Who do I call during the day and overnight?

Ask the team to replace the example numbers on your card with the correct daytime, overnight and backup contact routes before you leave.

When will recovery and response be reviewed?

Agree the follow-up appointments and any scans with your team. This regimen does not set a universal scan or tumour-marker timetable.

> **Try explaining the plan back:** Explain back your chemotherapy days, how radiation is scheduled separately, the tests before treatment and whom you will call if unwell.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin Day 1 · high emetic risk

Active medicines: Cisplatin + Etoposide

Emetogenic risk: High

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1;80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) then daily Reference course; check interactions.
Ondansetron	16 mg orally	Day 1 Review QT risk and the whole multiday prescription.
Dexamethasone	12 mg orally Day 1;8 mg daily Days 2–4	Reference high-risk NCIS course Reconcile all steroids; no duplicate orders.
Olanzapine	5 mg orally daily Days 1–4	Reference high-risk NCIS course Review sedation and additive extrapyramidal risk with metoclopramide.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days The high-risk row does not specify an end day/PRN qualifier; prescriber specifies duration and suitability.

Additional phase-specific support

- Pre/post cisplatin hydration under local policy. Etoposide1000mL may supply prehydration; oral Day1 etoposide cannot.
- Source suggested posthydration:10mmol magnesium sulphate and20mmol potassium chloride in1000mL saline over2hours; electrolytes/volume must be checked against patient status and local infusion policy.
- Routine furosemide is not advised unless fluid overload; mannitol only according to local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 2–3 · match actual drugs and route

Active medicines: Etoposide

Emetogenic risk: Route / schedule dependent

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCIS multiday guidance: give antiemetics appropriate to each day and for two days after. Continue the reconciled delayed cisplatin course; do not add another complete Day1 course.
- intravenous (IV) etoposide has low emetic risk. Oral etoposide requires the oral-route plan; split cisplatin means cisplatin is active on all three days. Pharmacy must issue one exact day-by-day prescription.
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by this NCCP document; additional prevention follows assessed risk/local policy.
- Assess eating/swallowing and refer for nutrition/pain support as needed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Only when carboplatinAUC5 substitution is prescribed

Active medicines: Carboplatin + Etoposide

Emetogenic risk: High carboplatin AUC≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1;80 mg orally Days 2–3	NCIS carboplatinAUC≥4 reference
Ondansetron	16 mg orally	Day 1
Dexamethasone	12 mg orally Day 1;8 mg daily Days 2–4	NCIS carboplatinAUC≥4 reference
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days Specify duration and suitability.

Additional phase-specific support

- This replaces the cisplatin support pathway; do not stack both high-risk regimens.
- Verify carboplatin renal dose independently; reconcile continuing etoposide route and multiday cover.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hearing / nerves

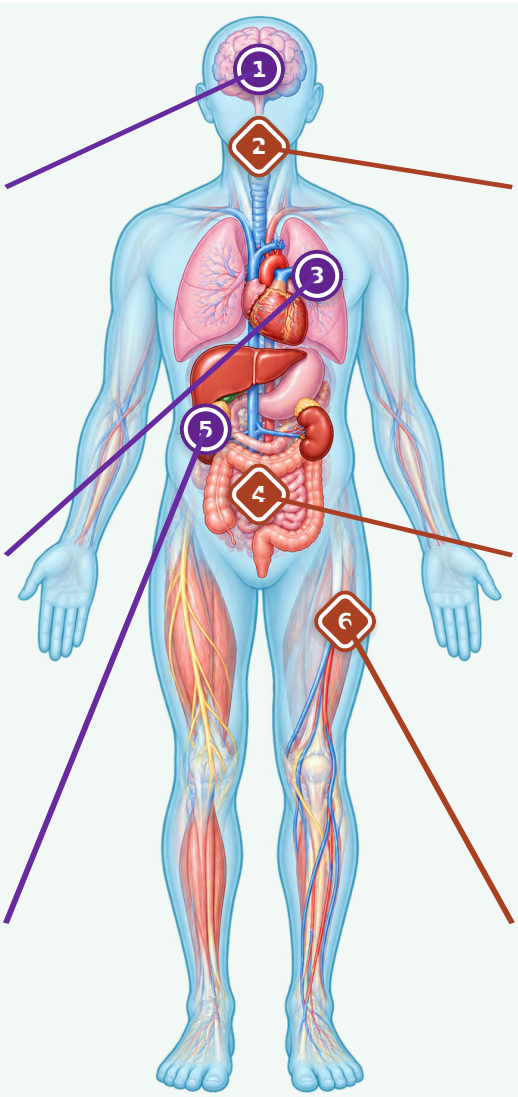
Cisplatin

Report ringing, reduced hearing, numbness or weakness before the next dose.

2 Mouth / swallowing

Treatment and chest radiation

Soreness may affect food and fluids. Contact the team early; inability to drink is urgent.



3 Breathing

Chemoradiation

New or worsening breathlessness needs urgent assessment, including after treatment finishes.

4 Sickness / appetite

Chemotherapy

Follow the anti-sickness plan. Call if vomiting continues or fluids will not stay down.

5 Kidneys / fluid balance

Cisplatin

Blood tests and fluids protect kidneys. Report much less urine or poor intake.

6 Blood cells / IV site

Chemotherapy

Fever or sudden illness can be serious. Report bleeding or infusion pain promptly.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Cisplatin + etoposide with radiotherapy · National Cancer Control Programme (NCCP) 00279

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00279 v8 · official regimen**
   https://assets.hse.ie/media/documents/279_CISplatin_and_Etoposide_and_radiotherapy (RT).pdf
2. **NCCP NCIS medical-oncology antiemetics v8**
   https://assets.hse.ie/media/documents/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **Etoposide capsules manufacturer product information**
   https://www.medicines.org.uk/emc/product/10533/smpc
4. **Turrisi et al.1999 · limited-stage small-cell lung cancer (SCLC) radiotherapy comparison**
   https://www.nejm.org/doi/full/10.1056/NEJM199901283400403